

PATIENT / FAMILY CARE STUDY
(NURSING PROCESS APPROACH)

OF A PATIENT WITH A DIAGNOSIS OF

GASTROENTERITIS

WITH

ACUTE LEFT LOBAR PNEUMONIA

AT KORLE-BU TEACHING HOSPITAL

WRITTEN BY

ERIC OFFEI AFFEDZIE

OF SCHOOL OF NURSING, COLLEGE OF HEALTH SCIENCES

UNIVERSITY OF GHANA, LEGON

IN PARTIAL FULFILMENT OF REQUIREMENT FOR THE AWARD OF
PROFESSIONAL LICENSE BY THE NURSES AND MIDWIVES COUNCIL OF
GHANA

JULY 2011

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PREFACE

The Patient/Family Care Study is a detailed written report of nursing care rendered to a nursing client—an individual and his family within a specific period of time. It explores nursing care rendered from the time of encounter to termination of nurse-patient relationship. It gives an in-depth description and explanation of how a patient's response to a specified disease condition is diagnosed and given intervention.

The Patient/Family Care Study involves a record nursing care, documenting the problems of a nursing client and how they are dealt with by the nurse in the course of finding solution to the problems. It provides a systematic way of collecting data, analyzing information, and reporting the results of nursing care.

This Patient/Family Care Study is based on the concept of holistic care, taking into account all factors impinging on the health of the individual. It includes a study of the interaction between the patient, the family, the community and the health team. It is done using the nursing process approach.

This care study is carried out in partial fulfillment of requirement for the award of professional license by the nurses and midwives council of Ghana. It is an integral part of the curriculum for educating nursing students hence a prerequisite for completing the nursing course.

Care study offers the nursing student the opportunity to combine classroom academic work with clinical study of the practices of the nursing profession. It encourages learning by doing, the development of analytical and decision-making skills as well as reporting skills. Being based on the nursing process, the students become familiar with the use of the nursing process as a basis for practice thereby encouraging evidence based nursing care.

ACKNOWLEDGEMENT

All praise and thanks be to God, the Father of Mercy who has given me the enablement in spite of all challenges to commence and conclude this care study successfully.

My warmest gratitude is reserved for Mr Edward Owana, my care study patient. Without his consent to be studied, this care study would never have materialised. Not forgetting his wife Gifty for her affable cooperation and support throughout the period of the study.

Exceptional thanks go to the Matron and nursing staff of Korle-Bu Medical Ward II. They ensured continuity of care for my patient and gave me support and morale for this care study. The supporting staff and colleague students whom I worked with on Medical Ward II have not been forgotten for diverse manners of help.

Thanks go to my supervising tutor Mr Attiogbe. His persistent guidance has ensured the successful completion of this study.

And to my parents, Mr. Amos Offei Affedzie and Felicia Arthur who have ensured my coming thus far, I can't thank you enough. You taught me that, by mustering the force of inner will and determination, I can convert challenges into achievements.

Finally, I acknowledge and thank all authors and publishers whose works have been used as references in this care study. Gracias!

INTRODUCTION

“The unique function of the nurse is to assist the individual sick or well in the performance of those activities contributing to health or its recovery (or to peaceful death) that he would perform unaided if he had the necessary strength, knowledge or will, and to do so in such a way as to help him regain independence as quickly as possible”—Virginia Henderson, 1966.

The rationale behind a care study is to assist a patient to regain health (or peaceful death) and present a report of that assistance giving account of problems that were identified and how solutions were worked out from a nursing process perspective. Presented in this care study is a report of nursing care rendered to Mr. Edward Owana who was diagnosed of gastroenteritis with and then later with acute left lobar pneumonia. He was admitted to the medical ward II of the Korle-Bu Teaching Hospital and stayed on the ward for one week

At the beginning of the afternoon shift, on Wednesday 17th November 2010, Mr. Edward Owana arrived on the ward on a stretcher accompanied by his wife Gifty and an orderly from Surgical Medical Emergency (SME) Unit where he had already been detained for 3 days. He was fairly ill, weak and with a unsatisfactory level of hydration and but fully conscious with a persistent non-productive cough. Happening to be at the nurse’s station with the sister in charge at the time of his arrival, I was subsequently charged with the responsibility to carry out his admission to the ward. Thus began my therapeutic relationship with the patient that has resulted in this care study. Nursing care spanned about one week from the time of admission to the ward till discharge on 22nd November. His condition at the time of discharge was satisfactory. Interaction with Mr. Edward Owana and his family continued after discharge with home visit and regular phone communication till nurse-patient relationship was terminated finally on 9th January 2011.

This care study report has been organized into five chapters in line with the five phases of the nursing process. Chapter one deals with assessment of Mr. Edward Owana and his family. This involves collection of data about the patient to identify his problems. Data collected for assessment includes biographical data, developmental, past and present medical history, the family's medical and socioeconomic history as well as the patient's lifestyle and hobbies. An account is also made of the admission of the patient. Literature review on gastroenteritis and pneumonia as well as validation of data is also discussed.

Chapter two deals with analysis of data. A comparison is made between the signs and symptoms experienced by the patient and those obtained in literature review. Diagnostic investigations, clinical manifestations and pharmacology of drugs are analysed in tabular form. Causes of illness, treatment and complications are also discussed. Data is analyzed to arrive at appropriate nursing diagnosis reflecting the patient's response to actual or potential health problems.

Chapter three comprises the planning phase of the nursing process and has the tabulated plan of care for the stated nursing diagnoses spanning the objective criteria, nursing orders, intervention and evaluation.

Chapter four tackles the actual implementation of the care plan giving summary descriptions of activities which were undertaken from the moment of first contact with the patient at the time of admission to the ward till discharge and subsequent follow up with home visit.

In chapter five, evaluation of nursing care given to the patient and his family from encounter till termination of nurse-patient relationship is discussed. A summary and conclusion then ends this care study report by reviewing thematic issues that arose in the care study from admission to last home visit after discharge.

CHAPTER ONE

1.0 ASSESSMENT OF PATIENT AND FAMILY

Nursing assessment begins the nursing process with appraisal of the health status of the patient. Through observation, questioning and examination, data about the patient and his family is gathered and analyzed. This chapter documents pertinent data obtained during interaction with Mr. Edward and his family at the assessment phase of the nursing process. It entails biographical data, developmental, past and present medical history, the family's medical and socioeconomic history as well as the patient's lifestyle. Literature review on gastroenteritis and pneumonia as well as validation of the data obtained is also discussed.

1.1 PATIENT'S PARTICULARS

Name: Mr. Edward Owana

Age: 43years

Sex: Male

Marital status: Married

Date of Birth: 1st June 1967

Place of Birth: Korle Gonno

Home Town: Kwahu Nkwatia

Ethnicity: Krobo

Language Spoken: Ga, Twi, English,

Nationality: Ghanaian

Religion: Christian

Educational Background: Middle School

Occupation: 'trotro' driving

Residence: PT 5, Agape

Next of Kin: Gifty Agbonyovi

Relationship: wife

Address of next of kin: PT 5, Agape

Patients Diagnosis: Gastroenteritis and acute Left lobar pneumonia

Date of Admission: 17th November 2010

Date of Discharge: 22nd November 2010

Ward: Medical ward II

Hospital: Korle-Bu Teaching Hospital

Medical/Surgical Team: Team C

Folder Number: 125759

NHIS Number: Not Registered

Height: not measured

Weight: 69.5 kg

FAMILY MEDICAL/SOCIO-ECONOMIC HISTORY

Mr Edward Owana is not aware of any familial disease such as diabetes mellitus, hypertension, asthma or sickle cell disease in his mother's or father's side of the family or among his maternal or paternal siblings. Neither he nor his wife and children are registered with the National Health Insurance Scheme (NHIS). They have poor health seeking behaviour as they scarcely go to the hospital and usually resort to over the counter drugs such when feeling unwell according to the patient. Mr Edward Owana lives with his wife and two school going children (a 6year old girl and 11year old boy) in a rented residence with other tenants. His wife Gifty is a food vendor, but according to her, that job is not profitable in the area where they live so she has suspended it for a while and is currently unemployed. Mr Edward therefore solely shoulders every financial

responsibility. As a ‘trotro’ driver his daily sales is variable. The family lives on what he brings home each day such that whenever he is unable to go to work, there isn’t enough for the next day. This was a major source of anxiety for him during his stay on the ward. He said the children would be unable to go to school as there would not be pocket money for them and virtually nothing at home for them to feed on as he was not at work. He was therefore wanted to be discharged home as quickly as possible.

1.2 PATIENT’S DEVELOPMENTAL HISTORY

Mr. Edward Owana comes from a family of 8 children of one mother but different fathers. He is the second child of three of his father and grew up with his paternal aunt. He says he hardly knows his own mother as he was supposedly taken away from her at about 3-4 months of age and nursed by her aunt instead. He was therefore weaned much earlier than usual. His immunization schedule was duly completed and he grew up without any deformities.

1.3 PATIENT’S LIFESTYLE / HOBBIES

As a trotro driver Mr. Edward Owana leaves home around 5:30 am after his early morning routine of oral hygiene with toothbrush and toothpaste and bath with tap water. He does not eat before leaving home. He eats much later, food bought from food vendors while at work in his car or at bus stops. He usually returns home after 10:00pm to eat in the house. He works about 17 hours a day, 6 days in a week with Sunday as his only off day. He hardly has enough time to pay attention to his health and eating habits. He says he occasionally drinks alcoholic beverages such as beer especially on Sundays with friends but does not smoke.

1.4 PAST MEDICAL HISTORY

According to Mr. Edward Owana, he has been admitted to hospital only once for swollen hands and feet. This occurred about 7 years ago in Libya where he was practicing his career as an auto

mechanic. He spent only three days on admission. He is not on any prescribed medication. He treated himself for malaria about a month ago with “Class Malacure” and “FPAC” which he usually takes when having occasional feverish conditions or headaches. He also usually takes a medication known to him as ‘G’ (that is Aludrox) for occasional abdominal upsets. He gets these from pharmacy shops. He has no known allergies.

1.5 PRESENT MEDICAL HISTORY

On 12th November Mr Edward was apparently well when leaving home for work. Later in the day, he felt headache and generalised body pains while at work and was taken to a clinic by his mate. Blood film microscopy showed of malaria parasites (mps) 2⁺⁺. He was given medications and taken home. Later that night he complained of stomach pain and had about four watery stools with vomiting. This resulted in a state of collapse for which he was rushed to a nearby maternity home. Blood Pressure was checked and recorded as 90/40 mmHg. He was resuscitated with 1L Ringers Lactate and 500mls Normal saline intravenous. He was also given IV GVithier 160 mg stat. After stabilisation the client was referred to Korle-bu Teaching Hospital (KBTH) for expert management.

Assessment reveals that the stomach pain was colicky in quality, centrally located in the abdomen and was non-radiating with no identified relieving or aggravating factors. It started around 11pm followed by three episodes of non-projectile vomiting which expelled food previously eaten but was non-bloody and contained no bile. Nature of the watery stools was not noted except that there were about four episodes. Inquiry about food eaten revealed that the patient had ‘koko’ with ‘kose’ for breakfast and jollof rice with egg for supper. These he bought from vendors.

At the time of his admission to the ward, the patient presented with coughing and complaint of pain in the left aspect of the chest. Inquiry revealed that the pain was sharp and agravated by

taking in breath or coughing. It was non-radiating. The patient manifested facial grimaces upon movement, restlessness and Irritability. He said that, at the Surgical Medical Emergency Unit, his bed was directly under a fan and near a window where he was exposed to much cold.

1.6 ADMISSION OF PATIENT

The client arrived on the Medical Ward II on 17th November 2010 at 2:30pm, on a stretcher accompanied by his wife Gifty and an orderly, being admitted for Dr. Akpah through Surgical Medical Emergency (SME) unit of Korle-Bu Teaching Hospital where he had been on detention since 14th November, 2010 with provisional diagnosis of Gastroenteritis with history of watery stools, vomiting and abdominal pain three days prior. During his detention at the Surgical Medical Emergency, he got a Respiratory Tract Infection (RTI) for which diagnostic investigations was being ordered. Condition on admission at the medical ward II was ill. He was inadequately hydrated and weak but fully conscious with a persistent non-productive cough.

The patient's identity was verified by mentioning his name for response. He was then welcome and immediately admitted and made comfortable in a simple bed with head end of the bed raised about 35 degree to relieve difficulty in breathing. Vital signs were then checked and recorded: Temperature: 36 degree Celsius, Pulse: 84beats per minute, Respiration: 25cycles per minute and Blood Pressure: 140/90mmhg. Plan of treatment was to undertake series of investigations including urine R/E and C/S, LFT, stool R/E and chest X-ray. Patient was to be kept on strict intake and output and managed on IV Ciprofloxacin 200mg, 12hourly×24hours; Tab Coartem four tablets, twice daily for three days; IV Metronidazole 500mg 8hourly×24hours; IV Nexium 40mg stat, IV Augmentin 1.2g 8hourly for 5 days and Cap Tramadol 500mg three times daily for 5 days.

The patient's name was entered into the admission and discharge book as well as the daily ward state. He was told where the washroom could be located and his wife was asked to buy a cup so

to be used to measure his intake and a pail for collecting urine to be measured as he was not on urinary catheter. Strict intake and output monitoring was indicated on the patient's folder and urine and blood samples taken for lab investigations as ordered. Later the client's informed consent to be the patient for this care study was requested. How the project would be done from commencement to termination including home visits was explained to which he agreed.

1.7 PATIENT'S CONCEPT OF ILLNESS

Mr. Edward believed his condition has a spiritual cause. According to him, he wasn't brought up by his mother as he was taken away from her at only three months old and has never related cordially with her. He said his mother does not like him and does not want him to succeed in life hence has caused his lack of success in his career as an auto mechanic and is now adding this sickness. He also said that he had been informed of a prayer camp which he will attend immediately after being discharged. Also he blamed nurses that, it was because they put him directly under a fan and near a window at the Surgical Medical Emergency Unit that he had got a cough. He thought the cough was due to the cold condition he was exposed to.

1.8 LITERATURE REVIEW ON DISEASES

This section deals with documented information about the conditions Edward was diagnosed with, that is gastroenteritis and left lobar pneumonia.

1.8.1 GASTROENTERITIS

Gastroenteritis refers to inflammation of the stomach lining and intestines accompanying numerous gastro-intestinal disorders. It is an acute infectious syndrome characterized by diarrhoea, vomiting, and abdominal cramps. It is also known as enterogastritis, stomach flu or gastric flu. Until fairly recently Gastroenteritis was not considered as a discrete diagnosis.

INCIDENCE

According to Parashar , Hummelman, Bresee, Miller & Glass (2003) every year worldwide rotavirus in children under 5, causes 111 million cases of gastroenteritis and nearly half a million deaths of which 82% occur in the world's poorest nations. In 1980 gastroenteritis from all causes caused 4.6 million deaths in children with most of these occurring in the third world (Mandell 2004). The incidence in the developed world is as high as 1-2.5 cases per child per year and is a major cause of hospitalization in this age group. Adults tend to have milder cases of the illness than do children and the very old.

ETIOLOGY

Gastroenteritis has many causes. Viruses and bacteria are the most common. Those caused by viruses may last one to two days but bacterial cases can continue for longer periods of time. Chemical toxins, most often found in seafood, food allergies, heavy metals, antibiotics, and other medications may also cause Gastroenteritis. In up to 50% of diarrheal outbreaks, no specific agent is found.

Viral gastroenteritis, is perhaps the most common type of diarrhoea in the world caused rotaviruses, caliciviruses, Parvoviruses - The human bocavirus (HBoV), Norwalk viruses, and adenoviruses. Norovirus causes 50-70% of cases of gastroenteritis in adults making it the most common cause of gastroenteritis. Adenoviruses commonly cause respiratory illness; but can also lead to gastroenteritis. Astrovirus infection is the third most frequent cause of gastroenteritis in infants.

Bacterial gastroenteritis may be caused by *Staphylococcus aureus* commonly found in food poisoning . *Escherichia coli* infections such as *E. coli* O157:H7 causes complications of gastroenteritis. *Salmonella* (*S. typhimurium* and *S. enteritidis*) which cause Salmonellosis is

contracted by ingesting contaminated food or water. Campylobacter occurs by the consumption of raw or undercooked poultry meat and is also associated with unpasteurized milk or contaminated water. Shigella bacteria spreads from eating contaminated food, drinking contaminated water and from an infected person to another person. Clostridium difficile is associated with longterm antibiotic therapy on clindamycin , fluoroquinolones, ciprofloxacin penicillins, and cephalosporins as well as hospitalization.

Parasites and Protozoans are less frequently responsible for intestinal irritation. Giardia is the most frequent cause of waterborne diarrhoea known as giardiasis.

PATHOPHYSIOLOGY

Microorganisms cause gastroenteritis by secreting toxins that stimulate excessive water and electrolyte loss, thereby causing diarrhoea, or by directly invading the walls of the gut, triggering inflammation that upsets the balance between the absorption of nutrients and the secretion of wastes. The onset may be slow but more often it is abrupt and virulent with rapid loss of fluids and electrolytes caused by persistent vomiting and diarrhoea. Hypokalaemia and hyponatraemia, acidosis and alkalosis may develop. The conditions may be attributable to bacterial endotoxins, bacterial or viral invasion, chemical toxins or miscellaneous conditions such as lactose intolerance. The severity of gastroenteritis varies from a sudden but transient attack of diarrhoea to severe dehydration. There may be noninflammatory infection of the upper small bowel, or inflammatory infections of the colon. The condition is usually of acute onset, normally lasting 1–6 days, and is self-limiting.

CLINICAL MANIFESTATIONS

Common gastroenteritis symptoms include Low grade fever to 37.7 C Headache ,Nausea with or without vomiting, Mild-to-moderate diarrhoea , abnormal flatulence, crampy abdominal pain The cramps may come in cycles, increasing in severity until a loose bowel movement occurs and

the pain resolves somewhat. Bloody stools may occur infection in amoeba, Campylobacter, Salmonella, Shigella or Escherichia coli infection. More serious symptoms of gastroenteritis include blood in vomit or stool ,vomiting more than 48 hours ,Fever higher than 40 C, Swollen abdomen or abdominal pain, lethargy and dehydration.

INVESTIGATIONS FOR MEDICAL DIAGNOSIS

Diagnosing gastroenteritis is based on symptoms, a complete medical history and physical examination. History taken include travel history, exposure to poisons or other irritants, diet change, food preparation habits or storage and medications as to whether the patient has been using broad-spectrum or multiple antibiotics in their recent past. The duration, frequency, and description of the patient's bowel movements and if they experience vomiting are also noted.

No specific diagnostic tests are required in most patients with simple gastroenteritis. If symptoms including fever, bloody stool and diarrhoea persist for two weeks or more, examination and cultures of stool to identify the causative bacteria or microscopy for parasites ova and cysts may be done. When the symptoms are not enough to diagnose gastroenteritis, several tests may be performed in order to rule out other gastrointestinal disorders. These include rectal examinations, complete blood count, electrolytes and kidney function tests.

TREATMENT

The treatment of gastroenteritis depends on the cause and the severity of symptoms and may include antibiotics or simply supportive care.

Medical management: The objective of treatment is to replace lost fluids and electrolytes and to treat underlying cause where the specific causative organism is identified. Medications used in the treatment of gastroenteritis include antiemetics such as Metoclopramide and Ondansetron. Antibiotics like Fluoroquinolones or macrolides are used if symptoms are severe or a susceptible bacterial cause is isolated or suspected. Pseudomembranous colitis, usually caused by antibiotics

use, is managed by discontinuing the causative agent and treating with either metronidazole or vancomycin.

Antimotility drugs such as Loperamide is commonly used for the symptomatic treatment of diarrhoea but is discouraged in people with bloody diarrhoea or diarrhoea complicated by a fever and children as it may cross the immature blood brain barrier and cause toxicity. Bismuth subsalicylate (BSS), is also used in mild-moderate cases. Antispasmodics such as Butylscopolamine (Buscopan) are useful in treating crampy abdominal pain.

Nursing management of patient: Aim of treatment is to rehydrate, maintain nutrition and personal hygiene and prevent dehydration. The primary treatment of replenishment of water and electrolytes lost through diarrhoea is by administration of intravenous fluids or by giving the person oral rehydration therapy (ORT). Dietary concerns include avoidance of foods high in simple sugars such as soft drinks and juice as the osmotic load might worsen diarrhoea. After the acute phase, water may be given by mouth. If it produces no vomiting or diarrhoea or vomiting, clear fluids may be added followed by a diet of foods that appeal to the patient if tolerated. Supportive Treatment employs bed rest, and sedation.

Complications: Dehydration is a common complication of diarrhoea. It may be characterised by dry mucous membranes, reduced skin turgor, skin discoloration, sunken fontanelles in children, sunken eyeballs, light-headedness and decreased fluid output. This may set up a cascade which result in renal failure, poor perfusion and ultimately shock.

1.8.2 PNEUMONIA

Pneumonia is an inflammatory condition of the parenchyma of the lung alveoli and abnormal fluid retention in the alveolar spaces.

INCIDENCE

Pneumonia is a common illness in all parts of the world. It is a major cause of death among all age groups and is the leading cause of death in children in low income countries. The World Health Organization estimates that one in three newborn infant deaths is due to pneumonia. It is estimated that up to 1 million of the over two million children under five who die each year worldwide are caused by *Streptococcus pneumoniae*, and over 90% of these deaths take place in developing countries. Mortality from pneumonia generally decreases with age until late adulthood with increased mortality in the elderly. Furthermore Pneumonia occurs more commonly in males than females, and more often in Blacks than Caucasians due to differences in synthesizing Vitamin D from sunlight. Individuals with underlying illnesses such as Alzheimer's disease, cystic fibrosis, emphysema, tobacco smoking, alcoholism, or immune system problems are at increased risk for pneumonia.

ETIOLOGY AND CLASSIFICATION

Pneumonia has a classification system that is closely related to its etiologic factors. The primary system of classification is the combined clinical classification system which combines factors such as age, risk factors for certain microorganisms, the presence of underlying lung disease or systemic disease, and whether the person has recently been hospitalized. Other classifications are based on the anatomic changes that can be found in the lungs during autopsies, microbial cause, and a radiological classification. Pneumonia can be due to microorganisms, irritants or unknown causes. The most common causes of pneumonia are viruses and bacteria. Less common causes of infectious pneumonia are fungi and parasites.

Clinical classification of pneumonia comprises "acute" and "chronic" pneumonias. Chronic pneumonias tend to be either non-infectious, or mycobacterial, fungal, or mixed bacterial infections caused by airway obstruction. Acute pneumonias are further divided into the classic

bacterial bronchopneumonias (such as *Streptococcus pneumoniae*), the atypical pneumonias (such as the interstitial pneumonitis of *Mycoplasma pneumoniae* or *Chlamydia pneumoniae*), and the aspiration pneumonia syndromes. Chronic pneumonias, on the other hand, mainly include those of *Nocardia*, *Actinomyces* and *Blastomyces dermatitidis*, as well as the granulomatous pneumonias (*Mycobacterium tuberculosis* and atypical mycobacteria, *Histoplasma capsulatum* and *Coccidioides immitis*).

The combined clinical classification scheme, identifies two broad categories of pneumonia : community-acquired pneumonia , hospital-acquired pneumonia . Community-acquired pneumonia (CAP) is infectious pneumonia in a person who has not recently been hospitalized and is the most common type of pneumonia. The most common causes of Community-acquired pneumonia (CAP) include, viruses, the atypical bacteria, *Haemophilus influenzae*. and *Streptococcus pneumoniae* -the most common cause of community-acquired pneumonia worldwide.

Hospital-acquired or nosocomial pneumonia is acquired during or after hospitalization for another illness or procedure with onset at least 72 hrs after admission. Up to 5% of patients admitted to a hospital for other causes subsequently develop pneumonia. It includes MRSA, *Pseudomonas*, *Enterobacter*, *Serratia* and ventilator-associated pneumonia.

Other types of pneumonia are Bronchiolitis obliterans organizing pneumonia (BOOP) also known as cryptogenic organizing pneumonitis (COP) caused by inflammation of the small airways of the lungs, Eosinophilic pneumonia caused by invasion of the lung by eosinophils, and often occurs in response to infection with a parasite or after exposure to certain types of environmental factors. Chemical pneumonia or pneumonitis) is caused by chemical toxicants such as pesticides, which may enter the body by inhalation or by skin contact. lipoid pneumonia involve an oil toxic substance. Aspiration pneumonia is that caused by aspirating foreign

objects which are usually oral or gastric contents, either while eating, or after reflux or vomiting which results in bronchopneumonia. Dust pneumonia is caused by excessive exposure to dust, which settles all the way into the alveoli of the lungs, stopping the cilia from moving and preventing the lungs from ever clearing themselves. Necrotizing pneumonia, includes pneumonias that cause substantial necrosis of lung cells, and sometimes lung abscess. Implicated bacteria are *Staphylococcus aureus*, *Klebsiella pneumoniae* and *Streptococcus pyogenes*. Opportunistic pneumonia includes those that frequently affects immunocompromised victims. Main pathogens are cytomegalovirus, *Pneumocystis jiroveci*, *Mycobacterium avium-intracellulare*, invasive aspergillosis and invasive candidiasis.

Other classification systems are focused on the anatomic or pathologic appearance of the lung and include lobar pneumonia involving a single lobe, or section, of a lung. Multilobar pneumonia involves more than one lobe, and it often causes a more severe illness. Bronchial pneumonia affects the lungs in patches around the bronchi or bronchioles while interstitial pneumonia involves the areas in between the alveoli, and is more likely to be caused by viruses or by atypical bacteria.

PATHOPHYSIOLOGY

Typically pneumonia occurs, when the causative microbe invades the cells lining the airways leading to cell death. When the immune system responds to the infection, even more lung damage occurs as white blood cells activate certain chemical cascades which result in leakage of fluid into the alveoli. This combination of cell destruction and fluid-filled alveoli interrupts the normal transportation of oxygen into the bloodstream.

Bacteria and virus typically enter the lung when airborne droplets are inhaled, but bacteria can also reach the lung through haematogenous spread another infected site of the body. The normal

flora of the nose, mouth and sinuses, and can also be inhaled into the alveoli. Parasites enter the body through the skin or by being swallowed and reach the lungs, usually through the blood. The immune system response depends on the type of causative microbe. Viral pneumonia involves activation of chemical cytokines by lymphocytes. Bacterial pneumonia involve neutrophils which release cytokines leading to fever, chills, and fatigue common in bacterial and fungal pneumonia. In parasitic pneumonia eosinophils, respond to parasite infection and can lead to eosinophilic pneumonia, thus complicating the underlying parasitic pneumonia.

CLINICAL MANIFESTATIONS

The signs and symptoms of lobar pneumonia has sudden onset with a high fever of 39° to 40°C and is often accompanied by shivering attacks (rigors). A severe pain in the chest over the affected lobe of the lung may be present. This pain characteristically catches the breathing so that the patient is nervous of taking a deep breath. For this reason also, cough which is usually present is short and suppressed. The pulse and respiration are usually rapid.

The face is flushed and herpes labialis (cold sores) is very commonly present on the lips and cheek. This is a small group of vesicles clustered together which after a few days crust and resolve. Herpes labialis is most commonly associated with lobar pneumonia or the common cold. After a day or so the cough becomes moist with the typical rusty sputum, the colour being due to altered blood. It may also produce greenish or yellow sputum. The sputum is very thick and tenacious and often adheres to the side of the sputum container.

In severe cases, the breathing may be very distressed and cyanosis is present. In these severe attacks delirium may also occur. Vital signs records of People with infectious pneumonia show fever or sometimes low body temperature, an increased respiratory rate, low blood pressure, increased pulse or a low oxygen saturation, as indicated by either pulse oximetry or blood gas analysis with resultant cyanosis, confusion and shortness of breath. They may cough up blood,

experience headaches, or develop sweaty and clammy skin. Other symptoms are loss of appetite, fatigue, nausea, vomiting, mood swings, and joint pains or muscle aches.

INVESTIGATIONS FOR MEDICAL DIAGNOSIS

Pneumonia is usually diagnosed based on symptoms and physical examination alone but further investigations such as Chest X-ray, blood tests and sputum cultures are ordered where the facilities are available to confirm the diagnosis.

Findings from physical examination often show decreased expansion of the chest on the affected side. Crackles are heard over the affected area during inspiration on auscultation. Percussion may be dulled over the affected lung, but with increased rather than decreased vocal resonance (which distinguishes it from a pleural effusion). While these signs are relevant, they are insufficient to diagnose or rule out pneumonia. Sputum cultures, blood cultures, tests on respiratory secretions, and specific blood tests are used to determine the microbiologic classification. Such laboratory testing typically takes several days, microbiologic classification is usually not possible at the time of initial diagnosis

Differential diagnosis involve further investigations to rule out diseases and/or conditions such as Chronic obstructive pulmonary disease (COPD) or asthma, Pulmonary edema bronchiectasis, lung cancer and pulmonary emboli which present with similar clinical features to pneumonia

TREATMENT

Medical management: Typically, oral antibiotics, rest, fluids, and home care are sufficient for complete resolution. Bacterial pneumonia is usually treated with Antibiotics such as amoxicillin, erythromycin, or azithromycin. Third and fourth-generation cephalosporins, carbapenems, fluoroquinolones, aminoglycosides, and vancomycin are often given intravenously and may be used in combination.

No specific treatments exist for most types of viral pneumonia. Influenza A may be treated with rimantadine or amantadine while influenza A or B may be treated with oseltamivir or zanamivir. These are beneficial only if they are started within 48 hours of the onset of symptoms

In chemical pneumonitis there is no evidence to support the use of antibiotics without bacterial superinfection. If infection is present in aspiration pneumonia, the choice of antibiotic include clindamycin, a combination of a beta-lactam antibiotic and metronidazole, or an aminoglycoside. Corticosteroids are commonly used in aspiration pneumonia, but there is no evidence to support their use either.

Nursing management

Nursing management aims at maximising respiratory capacity, relieving respiratory distress and conservation of energy and adequate nutritional status. The patient should be nursed in a well ventilated atmosphere but avoiding draughts. He should be propped up in bed in a comfortable upright position and encouraged to cough up sputum. If breathing is difficult and the patient is cyanosed, oxygen may be helpful by oxygen mask. To conserve bed rest should be encouraged. Diet should be light with plenty of fluids Measures should also be taken to prevent the spread of infection by encouraging the patient to cover his mouth when coughing.

COMPLICATIONS

Without proper treatment, pneumonia can lead to such life-threatening complications as septic shock, hypoxemia, and respiratory failure. The infection can also spread within the patient's lungs, causing empyema or lung abscess.

1.9 VALIDATION OF DATA

Information utilised in rendering care to the patient as reported in this care study has been gathered is from well informed sources and efforts were made to ensure that they are accurate

and valid. Subjective data was taken from the patient himself while objective data is obtained from significant others (the patient's wife) and various tests to identify the patient's problems and their sources. Data about the plan and progress of treatment as instituted by the physician team was collected from the patient's folder, as well as from direct discussion with them.

Literature reviews on the conditions were obtained from textbooks. Others were obtained through my own observation and questioning and examination of the patient.

CHAPTER TWO

2.0 ANALYSIS OF DATA

Analysis generally refers to the systematic examination of the nature or cause of a phenomenon. Data analysis is the second phase of the five step nursing process. Data gathered in the assessment phase of the nursing process needs to be analysed to enable the nurse to diagnose the presence of potential and actual health problems of the patient and how he responds to them. This chapter tackles comparison of the data collected about the patient with standard, exploration of the strength and weaknesses of the patient and his family as well as patient's health problems. It ends with statement of nursing diagnosis for problems identified.

2.1 COMPARISON OF DATA WITH STANDARD

A comparison is hereby being made between actual experiences of the patient and standard documented evidence to identify deviations. This includes the investigations requested by physicians for medical diagnosis; causes of the patient's illness, clinical manifestations of the patient's condition as well as pharmacological and non pharmacological management ordered.

2.2 DIAGNOSTIC INVESTIGATIONS

Series of investigations requested by physicians to diagnose Mr. Edward's condition included urine R/E and C/S, LFT, stool R/E and chest X-ray. A detailed account of the diagnostic investigations has been outlined in table of diagnostic investigations (table 1) on the next page.

TABLE I: DIAGNOSTIC INVESTIGATIONS

Series of investigations requested included urine R/E and C/S, LFT, stool R/E and chest X-ray.

DATE	SPECIMEN	INVESTIGATION	RESULT	NORMAL VALUES	INTERPRETATION	REMARKS
17/11/10	Urine	URINE R/E and CS				
		Macroscopy				
		Appearance:	Clear	clear	Normal	No treatment given
		Colour: light amber	Amber	Amber	Normal	No treatment given
		Chemistry				
		pH	6.5	4.5-8	Normal	No treatment given
		Blood	Negative	Absent	Normal	No treatment given
		Specific Gravity	1.025	1000-1.080	Normal	No treatment given
		Ketones	Negative	Absent	Normal	No treatment given
		Protein	Positive 2+	Absent	Abnormal	No treatment given
		Glucose	Negative	Absent	Normal	No treatment given
		Bilirubin	Negative	Absent	Normal	No treatment given
		Urobilinogen	Negative	Absent	Normal	No treatment given.
		Nitrites	Negative	Absent	Normal	No treatment given
		Leucocytes	Negative	Absent	Normal	No treatment given
		Bile pigment	Negative	Absent	Normal	No treatment given
		Bile salt	Negative	Absent	normal	No treatment given

DATE	SPECIMEN	INVESTIGATION	RESULT	NORMAL VALUES	INTERPRETATION	REMARKS
		Microscopy				
		Pus cells per HPF	< 10	<10	Indicates presence of infections	Antibiotics prescribed and served
		Epithelial cells per HPF	15	Absent	Normal	No treatment given
		RBC per HPF	0	Absent	Normal	No treatment given
		Crystals	Not seen	Absent	Normal	No treatment given.
		Cast	Not	Absent	Normal	No treatment given
		Yeast like cells	Present (2+)	Absent	abnormal	No treatment given
		S. haematobium	Not seen	Absent	Normal	No treatment given
		T. Vaginalis bacteria	Not seen	Absent	normal	No treatment given
17/11/10	Blood	LFT				
		Total BIL	0.9mg/dl	[0.2-1.3]	Within normal range	No treatment given
		Total Bilirubin (SI)	14.3umol/L	[3 – 22]	Within normal range	No treatment given
		Direct bilirubin	1.3umol/L	[0.2-5.3]	Within normal range	No treatment given
		AST	35.0 U/L	[15 – 46]	Within normal range	No treatment given.
		ALT	45.0 U/L	[13 – 69]	Within normal range	No treatment given
		Alb . phos	98.0 U/l	[38 – 126]	Within normal range	No treatment given
		GGT	90.0 U/L	[12 – 58]	Above normal range	Antibiotics prescribed and served
		Total protein	5.9g/dl	[6.3 - 8.2]	Within normal range	No treatment given

DATE	SPECIMEN	INVESTIGATION	RESULT	NORMAL VALUES	INTERPRETATION	REMARKS
		Total protein (SI)	60.0g/L	[63 – 82]	Within normal range	No treatment given
		Albumin	2.5g/dl	[3.5 – 5.0]	Within normal range	No treatment given
		Albumin (SI)	25g/L	35 – 50	Within normal range	No treatment given
17/11/10	Stool	STOOL CULTURE AND SENSITIVITY Salmonella Sp sensitive to penicillin	Positive	Negative	Confirms bacterial infection	Broad spectrum antibiotics prescribed and served
17/11/10		CHEST X-RAY	Consolidation of inferior aspect of left lung	Absence of consolidation	Confirms pneumonia	Antibiotics prescribed and served

2.3 CAUSES OF PATIENT ILLNESS

From the result of the diagnostic investigations, presenting signs and symptoms and history as well as literature review, it could be deduced that, Mr Edward initially presented at the hospital with gastroenteritis and later had a hospital acquired pneumonia. The gastroenteritis was attributable to bacteria infection most likely to be Salmonella contracted through consumption of contaminated food or water. Salmonella bacteria cause a type of gastroenteritis known as Salmonellosis contracted by consuming contaminated food or water. The hospital acquired left lobar pneumonia was attributed to Gram-negative bacterial infection. Mr. Edward was therefore treated with broad-spectrum antibiotics which is effective against all forms of bacterial infection including the gram negative bacteria.

2.4 CLINICAL FEATURES

Mr. Edward Owana complained of typical signs and symptoms of gastroenteritis such as abdominal pains accompanied by diarrhoea and vomiting, as well as those of left lobar pneumonia such as sharp pain in the left side of the chest and coughing. Details of the manifestations of gastroenteritis and left-lobar pneumonia exhibited by Mr. Edward are compared with those from literature review in the table below.

Table II: Comparison of Clinical Features Exhibited by patient with that of Textbook

Documented clinical features from text books	Clinical features presented by Mr. Edward Owana
Gastroenteritis	
Low grade fever to 37.7°C	Temperature on at admission: 36°C
Headache	Patient complained of headache
Nausea and vomiting	Patient had 3 episodes of vomiting
Mild-to-moderate diarrhoea	Patient has 4 episodes of watery stools

Documented clinical features from text books	Clinical features presented by Mr. Edward Owana
Abnormal flatulence Crampy painful abdominal pains with bloating Blood in vomit or stool Dehydration Heartburn	Abnormal flatulence was not observed in the patient Patient complained of severe abdominal pain but no bloating was observed Neither vomitus nor stool contained blood Patient showed signs of dehydration as manifested by weakness and lethargy Patient did not complain of heartburn
Pneumonia	
Vital signs records of People with infectious pneumonia show - Mild fever or sometimes low body temperature, - An increased respiratory rate, - Low blood pressure, - Increased pulse - Low oxygen saturation with Cyanosis and Confusion Shortness of breath They often have a cough producing rusty, greenish or yellow sputum High fever that may be accompanied by shaking chills A sharp or stabbing chest pain is experienced and worsened during deep breaths or coughs.	Vital signs of the patient at time of admission were recorded as Temperature: 36°C Respiration: 25cycles per minute (increased rate) Blood Pressure: 140/90mmhg(it was realised patient was hypertensive after his blood pressure was restored) Pulse: 84beats per minute. Oxygen saturation was not checked and no cyanosis was observed. Patient had mildly rapid breath Patient's cough was initially dry but later produced light brown mucus. Temperature was usually between 36°C and 38°C. The patient did not complain of chills Patient complained of sharp pains in the left side of the chest during inspiration

Documented clinical features from text books	Clinical features presented by Mr. Edward Owana
They may cough up blood	Haemoptysis was not observed
They may develop sweaty and clammy skin	Patient had sweaty and clammy skin
Loss of appetite, fatigue, and mood swings	Patient exhibited anorexia and fatigue. No
Joint pains or muscle aches.	manifestation of mood swing were observed but
	patient was anxious
	Patient complained of body pains

2.5 MANAGEMENT

Management of Mr. Edward Owana comprised of medical and nursing care. Medical management was undertaken with the following medications:

- a) IV Ciprofloxacin, 200mg 12hourly for 24hours
- b) Tab Ciprofloxacin, 500mg three times daily for 7 days
- c) IV Metronidazole, 500mg 8hourly for 24hours
- d) Tab Metronidazole, 400mg three times daily for 7 days
- e) IV Nexium, 40mg stat
- f) Tab Nexium 40mg daily for 7 days
- g) IV Augmentin, 1.2g 8hourly for 5 days
- h) 0.5 Dextrose Saline for 24hours
- i) IV Normal saline×24hours

- j) Tab Augmentin, 1g twice daily for five days
- k) IM Tramadol, 75mg stat
- l) Cap Tramadol, 50mg three times daily for five days
- m) Tab Paracetamol, 1g three times daily for five days
- n) Tab Lisinopril 20mg daily for seven days
- o) Tab Nifedipine, 20mg twice daily for seven days
- p) Tab Zithromax, 500mg daily for six days
- q) Tab Coartem, four tablets, twice daily for 3 days

Nursing care was mainly supportive. Fluids that were lost in diarrhoea and vomiting were replaced during the patient's detention at the SME unit. Rehydration continued on the ward with 0.5 Dextrose saline and 1L Normal Saline in 24 hours. Oral rehydration was also started along with maintenance of nutrition and personal hygiene

2.6 PHARMACOLOGY OF DRUGS

The patient was initially managed on parenteral medications from the day of detention at the Surgical Medical Emergency unit. His injections were later changed to oral tablets when he was admitted to the ward as outlined in Table III on the next page.

TABLE III: TABLE OF PHARMACOLOGY OF DRUGS FOR MR. EDWARD OWANA

Date	Drug	Dosage and route of administration	Classification	Desired effects of drug	Actual effect observed	Side effects/ Remedies	Remarks
14/11/10	IV Metronidazole	500mg 8hourly for 24 hours by intravenous infusion through a peripheral cannula	Broad spectrum antibacterial and antiprotozoal	Act as contact amoebicide in the digestive tract and tissue amoebicide in the liver, kidney and liver. Active against amoeba, trichomonas. Bactericidal on gram positive, gram negative and anaerobic organisms.	Used as a broad spectrum treatment in combination with ciprofloxacin to clear bacterial infection which caused the gastroenteritis	Gastrointestinal distress Dizziness Neutropenia Neurologic disturbance Metalic taste in the mouth	No side effects observed in the patient
(17/11/10)	Tab Metronidazole	400mg three times daily for 7 days orally					
14/11/10	IV Ciprofloxacin	200mg 12 hourly for 24hours by Intravenous injection	Antibiotic, Quinolone family, Fluroquinolone group	Bactericidal with double action of destruction of bacterial membrane and inhibition of bacterialDNA of both gram+ and gram- bacteria. Indicated for infections of	Combined with Metronidazole to clear bacterial infection	Nausea, vomiting diarrhoea, dyspepsia, abdominal pain Dizziness,	No side effects were observed in the patient

(17/11/10)	Tab ciprofloxacin	500mg thrice daily for 7 days orally	Antibiotic, Quinolone family, Fluroquinolone group	the upper and lower RTIs and gastrointestinal infections (but not for pneumococcal pneumonia)	Combined with Metronidazole to clear bacterial infection	sleep disorders, rash and pruritus. anorexia,	No side effects were observed in the patient
17/11/10	IV Nexium Tab Nexium	40mg stat, by intravenous injection. 40mg daily orally	Proton pump inhibitor	Acts as a Proton pump inhibitor which inhibit gastric acid secretion by blocking the hydrogen-potassium adenosine triphosphatase enzyme system (the 'proton pump') of the gastric parietal cell.	Used to control excessive secretion of gastric acid thereby reducing gastrointestinal irritation in the patient	Nausea, vomiting, abdominal pain, flatulence, diarrhoea or constipation, headache, and dizziness.	No side effects were observed in the patient
14/11/10	Tab Coartem	Full course oral administration	Antimalarial	To treat malaria in cases of uncomplicated infections due to Plasmodium or mixed infections including Plasmodium falciparum. Recommended for malaria	Patient had already treated for malaria but was ineffective. It was therefore used as a repeat treatment and cleared all malarial parasites		No side effects were observed in the patient

				cases with resistance to other anti-malarias			
15/11/10	IV Augmentin (Amoxicillin)	1.2g 8hourly for 5days by Intravenous injection	Antibiotic	A combination of amoxicillin and clavulanic acid-an irreversible lactamase inhibitor which produces stable non active complexes with enzymes and protects amoxicillin from degradation Indicated for the treatment of respiratory tract infections, by preventing invading bacteria from multiplying in the body	Effect was not satisfactory so reviewed and changed to Zithromax	Toxic epidermal necrolysis, exfoliative dermatitis, vasculitis prolongation of bleeding time, dizziness, headache,	No side effects were observed in the patient
15/11/10	IM Tramadol (Tramadol hydrochloride) Cap Tramadol	75mg stat by intramuscular injection 500mg three	Opioid Analgesic	Tramadol produces analgesia by two mechanisms: an opioid effect and an enhancement of serotonergic and adrenergic pathways.	Used to relieve patient of chest pains abdominal pains and Joint and muscular pain	Abdominal discomfort, diarrhoea, hypotension and respiratory depression as	No side effects were observed in the patient

		times daily for 5 days orally				well as addiction potential	
15/1/10	Tab Paracetamol	1000mg three times daily for 5 days Orally	NSAID. (Non- steroidal anti- inflammatory drug) Analgesic, Antipyretic	Has antipyretic, analgesic and anti-inflammatory effect. Counteracts inflammation by inhibiting prostaglandin synthesis	Used to relieve patient of chest and abdominal pain. Joint and muscular pain	Anaphylaxis hepato- toxicity haemolytic anaemia	No side effects were observed in the patient
19/11/10	Tab Nifedipine	20mg twice daily for 7days orally	Anti- hypertensive, Calcium channel blocker	Inhibit the flow of calcium across the membranes of smooth muscle cells causing smooth muscles to relax and diminishing the risk of muscle spasms	Patients blood pressure reduced from 140/90mmHg to 120/80mmHg	Hypotension, Peripheral oedema, palpitations, dyspnoea, headache	No side effects were observed in the patient
19/11/10	Tab Lisinopril	20mg daily for 7 days orally	Anti- hypertensive, angiotension converting enzyme inhibitor	Acts as protease inhibitor that blocks the formation of angiotensin II and decrease the degradation of bradykinin and other kinins. Cause a decrease in sodium and water	Patients blood pressure reduced from 140/90mmHg to 120/80mmHg	Tachycardia, palpitation, cerebrovascu- lar accident, myocardial infarction, Raynaud's	No side effects were observed in the patient

				retention, a decrease in blood pressure, improvement in cardiac output and a decrease in heart size		syndrome, confusion, mood changes, impotence	
19/11/10	Tab Zithromax (Azithromycin)	500mg daily for 6 days orally	Antibiotic	Interferes with DNA synthesis by suppressing formation of protein by bacteria, retards bacterial growth and cause death of microorganisms. Indicate for mild to moderate respiratory-tract infections; To treat a variety of infections, such as tonsillitis bronchitis, pneumonia,	Cleared the causative organism of the pneumonia	anorexia, dyspepsia, flatulence, constipation or diarrhea, dizziness, drowsiness, photosensitivity; rarely taste disturbances, tongue discoloration,	No side effects were observed in the patient

2.7 COMPLICATIONS

The patient showed signs of dehydration from the loss of fluids in watery stools and vomiting. This was however promptly remedied with administration of IV fluids (0.5 Dextrose Saline and ringers lactate).

2.8 PATIENT AND FAMILY STRENGTH

Patient and family strengths are those abilities and behaviours they exhibit which are favourable for specified nursing interventions. Nursing interventions are built on the patient and family strengths. The following strengths were noticed in the patient and his family.

1. Patient made personal effort to sustain breathing
2. Patient could tolerate prolonged sitting up in bed hence able to cope with the elevated head of bed
3. Patient made attempts to clear airway by coughing up small amounts of sputum
4. Patient's vomiting and diarrhoea had subsided by the time he was admitted to the ward
5. Patient could tolerate sips of fluids and small amounts of fluid diet
6. Patient showed good verbal communication about his pain
7. Patient and his wife showed interest in learning about his condition
8. Patient was ready to comply with treatment regimen so as to be discharged quickly
9. Patient could perform his own personal hygiene needs
10. Patient had satisfactory family support as he was visited regularly wife

2.9 HEALTH PROBLEMS

Some of the health problems of the patient that were identified include;

- a) Difficulty in breathing
- b) Dehydration

- c) Chest pains
- d) Abdominal Pain
- e) Persistent cough
- f) Weakness
- g) Loss of appetite
- h) Wrong perception of cause of the condition

2.10 NURSING DIAGNOSES

1. Breathing pattern, ineffective related to chest pain
2. Fluid volume deficit related to excessive loss of fluid through normal routes
3. Alteration in comfort (abdominal pain) related to increased peristalsis
4. Ineffective air way clearance related to presence of secretions (mucus)
5. Activity intolerance (active exercise) related to generalized weakness
6. Imbalanced nutrition: (less than body requirements) related to anorexia
7. Knowledge deficit (family) related to unfamiliarity with information about disease process and treatment

CHAPTER THREE

3.0 PLANNING FOR PATIENT AND FAMILY CARE

In the five step nursing process, planning is the third phase. It is the category of nursing behaviours in which strategies are designed to achieve the goals of care. Planning involves developing and modifying a care plan for the patient. To design the care plan, the nurse anticipates the patient's needs according to established priorities and involves the patient, his family and significant others to decide on outcome criteria to be achieved at the end of a specified period of time. Outlined in this chapter are the objective or outcome criteria for the care Mr. Edward Owana followed by his nursing care plan table.

3.1 OBJECTIVE / OUTCOME CRITERIA

These objectives were made in relation to their corresponding nursing diagnosis:

1. Patient will be able to breath normally within 2 hours as evidenced by:

1. Nurse observe patient's chest rise and fall at normal rhythm
2. Patient breathing within the normal rate of 15-22breaths per minutes
3. Having relaxed facial expression when taking deep breath

2. Client's fluid volume will return to normal levels during the period of hospitalization as evidenced by

- r) Patient having normal skin turgor as a sign of decreasing dehydration
- s) Increased urine output
- t) Blood pressure being restored to baseline

3. Patient will be relieved of abdominal pain within 24 hours as evidenced by:

1. Patient verbalizing absence of pain

2. Patient having relaxed facial expression
3. Nurse observe patient to change position in bed without clutching abdomen
4. Patient will be have patent airway for effective breathing as evidenced by
 1. Effective coughing which expels mucus
 2. Patient verbalizing no obstruction of airways
5. Patient will be able to engage in active exercise during stay on the ward as evidenced by;
 1. Patient being able to get out of bed and walking around the ward
 2. Patient being able to perform personal hygiene activities such as bathing without assistance
6. Patient will be able to eat well by the time of being discharged from the ward as evidenced by:
 1. Patient eating and 80 percent of meals served
 2. Patient verbalising that he can eat well to his personal satisfaction
7. Patient and his wife will have adequate knowledge about disease condition by the time of discharge as evidenced by:
 1. Patient and his wife being able to answer simple questions correctly about the disease condition
 2. Patient and his wife verbalising that they understand the actual cause of the condition
 3. Patient and his wife stating how they will prevent recurrence of the condition

TABLE IV: NURSING CARE PLAN FOR MR EDWARD OWANA

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATION STATEMENT	SIGNATURE
Wednesday, 17/11/2010 at 2: 45 pm	1. Breathing pattern, ineffective Related chest pain	1. Patient will be able to breath normally within 2 hours as evidenced by: 1. Nurse observe patient's chest rise and fall at normal rhythm 2. Patient breathing within the normal rate of 15-22 breaths per minutes 3. Having relaxed facial expression when taking deep breath	1. Monitor breathing pattern 2. Prop up patient in bed at an angle of 30° and encourage frequent position changes and deep-breathing 3. Stay with client and coach in taking slower more effective breaths 4. Assist client in the use of relaxation technique 5. Administer analgesics as ordered	1. Patient's breathe sounds, respiratory rate, depth and rhythm was assessed on arrival at ward. 2. Patient was propped up in bed by elevating the head of the bed at angle of 30°. This posture maximizes respiratory effort through greater lateral expansion of lungs 3. Patient was taught to lie in semi fowlers position and take slow long breath 4. It was explained to patient that he can make breathing more effective by consciously controlling it 5. 500mg Cap. Tramadol and 1g tablet paracetamol were administered as ordered.	Wednesday, 17/11/2010 at 3: 30 pm	Goal fully met as evidenced by observation that patient's chest was rising and falling at normal rhythm and respiratory rate of 22 breaths per minute	

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATION STATEMENT	SIGNATURE
Wednesday, 17/11/2010 at 3: 00 pm	2. Fluid volume deficit related to excessive loss of fluid through normal routes	2. Client's fluid volume will return to normal levels during the period of hospitalization as evidenced by a) Patient having normal skin turgor as a sign of decreasing dehydration b) Increased urine output c) Blood pressure being restored to baseline	1.Administer IV fluids as prescribed 2. Monitor and record blood pressure 4hourly 3.Encourage fluid intake by mouth 4.Strictly monitor Input and Output 5.Assess patient's skin turgor	1.IV Ringers Lactate was in-situ on arrival at ward and was continued 2. Blood pressure was checked and recorded 4hourly and compared with baseline. Range was 130/80- 140/90mmHg 3.Intake plan of 3-5 cups of water a day was discussed with patient 4. Patient was told to void into a pail and it was measured. All fluid intake were measured with a calibrated cup and recorded on the intake output chart 5.Skin turgor was assessed to monitors the status of hydration	Thursday, 18/11/2010 at 10: 30 am	Goal fully achieved as evidenced by : Patient having good skin turgor and blood pressure of 140/90mmHg. Intake was 410mls whiles output was 330mls	

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATION STATEMENT	SIGNATURE
Wednesday, 17/11/2010 at 4: 00 pm	3.Alteration in comfort (abdominal pain) related to increased peristalsis	3. Patient will be relieved of abdominal pain within 24 hours as evidenced by: 1. Patient verbalizing absence of pain 2. Patient having relaxed facial expression 3. Nurse observe patient to change position in bed without clutching abdomen	1.Assess patient for pain 2.Monitor and record vital signs 4 hourly 3.Teach patient relaxation techniques and how to use them to reduce pain. 4.Explain cause of the abdominal pain 5.Administer analgesics as ordered	1.Patient was assessed for pain intensity using a pain rating scale of 0 to 10 of which he indicated 5. Pain was location around the navel and was intermittent in nature 2.Vital signs were checked and recorded 4 hourly 3.Patient was advised to assume a comfortable posture that relieves the pain 4.It was explained to patient that inflammation results in release of body chemicals which cause increase in vascular permeability and causes pain felt in the abdomen. 5. 500mg Cap. Tramadol and tablet 1g paracetamol were administered as ordered	Thursday, 18/11/2010 at 8: 45 am	Goal fully met as evidenced by Patient the verbalizing absence of pain.	

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATION STATEMENT	SIGNATURE
Thursday, 18/11/2010 at 2: 45 pm	4. Ineffective air way clearance related to presence of secretions(mucus) in airway	4. Patient will be have patent airway for effective breathing by the time of discharge from the ward as evidenced by 1. Effective coughing which expels mucus 2. Patient verbalizing no obstruction of airways 3. Patient being relived of coughing reflex	1.Assess patient's respiration 2.Prop up patient in bed at angle of 30° 3.Instruct patient on the proper method of expel mucus effectively (controlled coughing) 4. Maintain adequate hydration	1.Checking and recording of patient's respiratory rate, depth and rhythm was done on arrival and 4 hourly afterward 2. Patient was propped up in bed by keeping head of his elevated 30° for patient to lie in semi fowlers position. This expanded chest cavity for easy breathing and coughing 3.Patient was taught effective coughing technique 4.Fluid intake of 3-5 cups of water was encouraged to thin secretions for easy expulsion a day was discussed with patient	Monday, 18/11/2010 at 8: 45 am	Goal fully met as evidenced by patient verbalising that he had relief from cough on Sunday	

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATION STATEMENT	SIGNATURE
Thursday, 18 /11/2010, at 8:30am	5.Activity intolerance related to generalized weakness	5.Patient will be able to engage in active exercise during stay on the ward as evidenced by; 1. Patient being able to get out of bed and walking around the ward 2. Patient being able to perform personal hygiene activities such as bathing without assistance	1.Teach energy conservation methods 2.Develop and adjust simple activity like brushing his teeth 3.Promote comfort measures on the activity 4.Cluster nursing care activities 5.Encourage bed rest	1.Patient was advised on the following energy conservation techniques: -To Sit rather than stand when performing activities such as brushing teeth -When performing task, rest every 3minutes for 5 five minutes. -Stop activity if fatigue is present 2. Patient was allowed to brush teeth in bed 3.Patient was made comfortable in bed 4.All related nursing procedures on patient was done together to reduce interruption of patient's rest period 5. patient was encouraged to stay in bed	Monday, 22/11/2010, at 9:00am	Goal fully met as evidenced by Patient being observed to walk to the washroom for elimination and bath without reporting feeling of tiredness	

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATION STATEMENT	SIGNATURE
Friday, 19/11/10 at 9:30am	6. Imbalanced nutrition: less than body requirements related to increased peristalsis	6. Patient will be able to eat well by the time of being discharged from the ward as evidenced by: 1. Patient eating and 80 percent of meals served 2. Patient verbalising that he can eat well to his personal satisfaction	1.Determine food preferences 2.Offer small frequent liquid diets 3.Offer easy to ingest and digestible meals as tolerated 4.Offer mouth care before and after meals 5.Eliminate offensive odors and sights before meals 6.Allow adequate time for patient to eat	1.Patient was asked which food he prefers to eat. His wife was told to bring light soup from the house for supper 2. Patient was encouraged to take small sips of liquid diets(akasa or tom brown) served at a time 3. Akasa or tom brown was usually served for breakfast. 4.Patient was encouraged to clean mouth before and after eating as he could do it by himself 5.Before eating, I ensured that his urinary pail was emptied and covered and the surrounding tidied up 6.The patient was allowed to eat at a pace which was convenient to him. Sometimes he left the food for a while and continued later	Monday, 22/11/10 at 9:30am	Goal fully met as evidenced by patient consuming more than 80%of all meals served	

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATION STATEMENT	SIGNATURE
Friday, 19/11/2010 at 9:00 am	7.Knowledge deficit(family) related to unfamiliarity with information about disease process and treatment	7. Patient and his wife will have adequate knowledge about disease condition by the time of discharge as evidenced by: 1. Patient and his wife being able to answer simple questions correctly about the disease condition 2. Patient and his wife verbalising that they understand the actual cause of the condition 3. Patient and his wife stating how they will prevent recurrence	1.Assess knowledge about condition 2.Educate patient and family about disease process and treatments 3.Ask patient and his simple questions about the disease conditions after educating them	1.Patient and his wife's knowledge on the condition was assessed 2. patient and his wife knowledge on were educated on: -Gastroenteritis, -Pneumonia and -Causes, risk factors, signs and symptoms -first aid management of vomiting and diarrhoea in gastroenteritis -Management of pneumonia 3. Patient and his wife were asked about causes, management and prevention of gastroenteritis and pneumonia	Friday, 19/11/10 at 9:30 am	Goal fully met as evidenced by patient and his wife answering questions correctly about the disease process and treatment	

DATE/ TIME	NURSING DIAGNOSIS (ND)	OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTIONS	DATE/ TIME	EVALUATIO N STATEMEN T	SIGNAURE

CHAPTER FOUR

4.0 IMPLEMENTATION OF PATIENT AND FAMILY CARE PLAN

Implementation follows planning and precedes evaluation thus forming the fourth phase in the five step nursing process. It is the stage at which the actions necessary for accomplishing the care plan are initiated and completed.

Outlined in this chapter is the summary of actual nursing care rendered to Mr. Edward Owana and his family throughout the period of his stay on the ward, preparation for his discharge and home visit for identification of resources in the home environment for social health or health assistance as needed by the client and his family.

4.1 SUMMARY OF ACTUAL NURSING CARE RENDERED TO MR. OWANA

Mr. Edward was detained at SME from Sunday 14th till Wednesday 17th of November. He arrived on the ward during the afternoon shift of Wednesday 17th, where I met him and effected his admission thus beginning my interaction with him. Nursing care subsequently continued till the following Monday when he was finally discharged but he left the ward on Tuesday, the 23rd of November. The nursing activities undertaken to assist him in the recovery of health is summarily reviewed in this section.

DAY OF ADMISSION, WEDNESDAY, 17TH NOVEMBER

Mr. Edward Owana arrived on the ward around 2:30 pm being admitted for Dr. Akpah through Surgical Medical Emergency (SME) Unit with diagnosis of Gastroenteritis and pneumonia. He presented with weakness and coughing and was brought in on a stretcher with a peripheral cannula in situ with IV Ringers lactate dripping well. The patient's identity was verified by mentioning his name for response. I introduced myself to him and his wife and welcome them to

the ward. They were given reassurance about the fact that the best possible care that would ensure recovery would be given.

The patient was then immediately made comfortable in an already made simple bed. He was propped up in the bed at an angle of 30° to lie in semi fowler's position to maximize lung expansion and relieve difficulty in breathing. Vital signs were then checked and recorded as; Temperature: 36 degree Celsius, Pulse: 84beats per minute, Respiration: 25cycles per minute and Blood Pressure: 140/90mmhg. Supplementary Oxygen set at 2mls per minute was provided through nasal prongs. The IV fluid bag (containing about 100mls of Ringers lactate) was remounted on a drip stand and ensured that the fluid was dripping well. Patient was taught take slow long breaths. He was directly monitored to note for respiratory abnormalities such as cyanosis.

The patient's name and other particulars were entered into the admission sheets which included the front index sheet, treatment sheet, continuation sheet, nurse's note, 4-hourly TPR sheet as well as input and output chart. The patient's name, age, diagnosis, date of admission was also entered into the admission and discharge book. The daily ward state was also completed. His wife was asked to buy a cup and a pail to be used to measure intake and output. He was told where the washroom could be located but was not actually taken there as he was weak and needed bed rest. Intake and output monitoring was started

Around 3:30 pm, patient was observed to relax in bed without exerting undue effort to breath. respiratory rate was about of 22bpm due to the interventions given

Patient also complained of transient feelings of abdominal pains. It was explained to patient that inflammation in the stomach and intestines causes the pain felt in the abdomen. He was therefore advised to relax in bed and assume a position that relieves the pain. Medications were served

around 6pm. Patient drugs were reviewed and IV Metronidazole and Ciprofloxacin were changed to their tablet form. Patient was told that the Cap. Tramadol tab paracetamol would relieve him of the pain

Vital signs were checked at 6pm and showed temperature-36.5°C, pulse 94, respiration 23cycles per minute and Blood Pressure: 140/90mmhg.

NURSING CARE FOR FIRST DAY ON ADMISSION, THURSDAY, 18TH NOVEMBER

According to the night nurses report the patient was able to sleep most of the night. His personal hygiene was maintained by himself. Breakfast of akasa was served of which patient ate very little.

Around 8am Patient complained of chest pains which was aggravated by coughing and taking deep breath. Despite the chest pains, patient had several bouts of coughs which produced small amounts of mucus. Patient was assessed for pain for intensity. The head of the bed was kept at about 30° to prop him up in bed with the rationale that it would improve chest expansion and minimizes irritation against the inflamed airways in order to reduce pain. This was explained to the patient. He was also taught effective coughing technique as follows:

To breathe deeply and slowly while sitting up as high as possible, then hold breath and cough forcefully from chest rather than throat using two short forceful coughs. This was initiated when patient has best pain relief and optimal level of awareness. Patient's respiratory rate, depth and rhythm was monitored

Patient also reported that he was feeling fatigued and weak .This was diagnosed as activity intolerance related to generalized weakness .He was taught energy conservation strategies such as sitting rather than standing when performing activities such as brushing teeth, resting every

3minutes for 5 five minutes when performing an activity and stopping activity if fatigue is present. All related nursing procedures on patient was done together to reduce interruption of patient's rest period. Patient was also encouraged to stay in bed Vital signs for the day ranged as follows: Temperature: 36.5—37°C. Pulse: 84—90 beats per minute. Respiration: 22—25cycles per minute. Blood pressure 140/90mmhg—130/80mmHg.

NURSING CARE FOR SECOND DAY ON ADMISSION, FRIDAY 19TH NOVEMBER

Patient appeared very optimistic with a cheerful facial expression and a positive attitude towards treatment. He verbalised absence of abdominal a pain but presence of a little chest pain. The night nurse had calculated his input which was 300mls while the output was 250mls. For breakfast, he was served with a cup of tom brown of which he ate only half. Mr. Edward was asked which food he prefers to eat. His wife was told to bring light soup from the house for supper. Prescribed drugs were also served.

During ward rounds, Mr. Edward seemed fairly well but was told by the doctors that they intended to monitor him for his response to the antibiotics until Monday, and if the assessment produced satisfactory result he would be allowed to go home. His antibiotic was changed from Augmentin to Zithromax. Tab Nifedipine and Lisinopril were added to his drugs

In the afternoon patient was educated about his Gastroenteritis, Pneumonia and Hypertension relating their causes, risk factors, signs and symptoms, first aid management of vomiting and diarrhoea in gastroenteritis as well management of pneumonia and hypertension. This was repeated when his wife was present at visiting time. In the evening patient had his light soup with fish for supper. Before eating, I ensured that his urinary pail was emptied and covered and the

surrounding tidied up. He drank about 80 percent of the soup. Medications were served as prescribed.

Vital signs for the day ranged as follows: Temperature: 36.5—36°C. Pulse: 87—90 beats per minute. Respiration: 22—25 cycles per minute. Blood pressure 130/90mmhg—12/80mmHg.

NURSING CARE OVER THE WEEKEND: SATURDAY, 20TH AND SUNDAY 21ST NOVEMBER

According to the nurses' notes, there wasn't any major incident over the weekend. Routine nursing care activities including, serving medications, checking of vital signs, Input and output monitoring and were continued. Mr. Edward undertook his own personal hygiene needs as he was feeling better and was ambulant. According to the patient his cough was relieved on Sunday.

PREPARATION OF PATIENT/FAMILY FOR DISCHARGE

Discharge planning constitutes activities that facilitate a client's movement from one healthcare setting to another. Its goal is to enhance continuity of care. Preparation of Mr. Edward and his wife for discharge spanned from his day of admission to the ward to the day he actually left the ward. The preparation was geared towards enabling him to recover health and prevent recurrence of condition. On the second day of admission, Mr. Edward was educated on the causes of gastroenteritis and how to prevent it. He was informed that the most important factor in avoiding gastroenteritis is food and water hygiene, and that there was the need to avoid buying and eating food from vendors and to verify that such food are prepared and served under good hygienic conditions if he really needed to buy them.

Mr. Edward was informed about the possible causes of pneumonia. He was told about the need to avoid dusty environments and smoking and to maintain personal hygiene as a preventive measure.

During ward rounds on Friday, 17th November, Mr. Edward was told that he was for possible discharge on the following Monday. This prompted him to start making preparations to settle his hospitals bills as he was not registered on the NHIS. He was formally discharged on Monday, 22nd November 2010 with bills amounting to GHC 280.36 (117.79 being expenses at SME and 162.57 being expenses at Medical Ward II). This was apart from expenses which he had already spent on drugs and investigations. Mr. Edward didn't have the money to pay his bills; he therefore had to stay on the ward until his bills were settled.

Whiles waiting, education about gastroenteritis and pneumonia was repeated to him and his wife. His wife was further advised to ensure that foods prepared at home were also done under good hygienic conditions. The importance of hand washing with soap under running water especially before eating and after visiting the toilet was emphasised. Also the need to ensure that leftover food from the previous night was well boiled before eating to avoid food poisoning was emphasised. They were also encouraged to visit the hospital whenever they were sick instead of resorting to self medication.

On Wednesday, 24th November 2010 Mr. Edward's bills were finally paid by his brother and he was ready to go home. He was taken through his drug regimen emphasising the right dose and time and was told that that it was needful for him to take all his drugs as directed even if he was feeling better to avoid recurrence. He was also informed of possible side effects of the drugs. He was informed that he had to come for review the next two weeks that is, Monday 6th December but he could come for any follow up or report any complaints even prior to the review date.

Finally I also informed them that although our meeting at the hospital was ending, I would visit them in their home as had already been agreed for continuity and follow up care. His discharge was recorded in the Admissions and Discharge book and the ward state form. I thanked them for their cooperation throughout the period of care. He left the ward with his wife around 11am.

4.2 FOLLOW UP, HOME VISIT AND CONTINUITY OF CARE

FIRST HOME VISIT

My first home visit was on Saturday, 20th of November. Mr. Edward was on the ward by then. The purpose was to get acquainted with their home environment and identify resources or problems in the home environment that could influence the health condition of the client and family. The family stays at Agape in the Ablekuma vicinity in Accra in a hired residence with other tenants. I got to the house around 14:30 pm. Mss Gifty came to meet me beside the road. On reaching the house, I was offered a seat outside their room on their veranda. I realised that the family of four lives in a one bedroom and hall apartment. This implies that there would be overcrowding.

Mr. Edward's chamber and hall accommodation was nestled between that of two other tenants. As I was facing the front part of the house I observed three windows covered with mosquito net. Their front door had a trap door whose mosquito net. Lightening could not be ascertained as I was outside the building and it was afternoon. Their main source of water was the pipe borne water which they fetch from another compound and keep in containers on their open veranda. This was not ideal as some of the containers uncovered and could easily admit dust. I told Mss Gifty that, this could result in contamination and result in the condition her husband was experiencing so she should ensure that the water is covered and kept clean all the time.

They do their cooking outside the room in an open space in front of the veranda. Although cooking in the open space was not ideal due to possible exposure to dust and the increased risk of contamination, I realised there was no other alternative space available. I did not meet the children as were not at home. I did not keep long as Mss. Gifty was preparing to go and see her husband at the hospital.

SECOND HOME VISIT

My second home visit was on the Sunday 28th November. I got to the house around 12:30pm and met only Mr. Edward at home. My first impression of him was of someone who had been through so much anxiety and still looked fatigued and in convalescence. I told them the purpose of my visit was to remind him of the review on next week Monday 6th December and also to find out how they are doing. I also enquired about his drugs and was informed that he was taking them well. I proceeded to talk about the need for him to pay more attention to his health as he seemed not to have a good health seeking behaviour. I entreated him to get himself, his wife and two children registered on the National Health Insurance Scheme so that they could access healthcare more regularly. Maintenance of good environmental and personal hygiene as an illness prevention was discussed. I informed him that my next visit would be my last visit and that he should not miss the review on Monday. I left the house around 3:15pm and as accompanied by Mr. Edward to the roadside to take a car back home.

REVIEW

On Sunday evening, 5th December, I called Mr. Edward to inform him that I would meet him at the hospital for his review on the next day. I therefore met him at Korle–Bu Teaching Hospital

around 9am on Monday, 6th December. He came to the hospital alone. I explained to him that the purpose of the review was to ascertain progress of treatment and address any complaints he might have. He said he had no complaints. I accompanied him to the ward to take his folder for the review with Dr. Akpah. According to Mr. Edward his chest sounds were assessed and he was advised by the doctor to come to the hospital if he had any problem and to make sure to finish his all drugs. I reminded him of my last home visit and the arrangement to be made with the Public health nurse in his area for continuity of care

THIRD HOME VISIT

Final home visit was on Sunday the 16th January 2010. Unfortunately I wasn't able to contact the appropriate public health nurse to undertake continuity of care for Mr. Edward Owana. I met only Mr. Edward as Mss. Gifty had gone to visit her sister with the children. After preliminary welcome message had been shared he informed me that he had finished his drugs and that he was feeling much better. He showed me the broth from boiled dandelion leaves which he was drinking twice daily. He said it was suggested to him by a certain herbal doctor who happened to be his passenger some days ago.

I suggested that it was better to go to the clinic when feeling unwell. I advised to be more proactive about his health regular visits to the hospital even for small conditions so that they would not become worse. I then reminded him that this would be my last visit but assured him that I would pass by anytime I was in the vicinity. I thanked him for their cooperation and he also showed gratitude for the care I rendered to him.

CHAPTER FIVE

EVALAUTION OF CARE RENDERED TO PATIENT BAND FAMILY

In the five step nursing process, evaluation constitutes the category of nursing behaviour in which the extent to which the established goals of care have been met is determined and recorded. It is considered the final phase of the nursing process but in practice, it is integral to effective nursing practice in all phases of the nursing process.

It is a continuous process and occurs concurrently with an on-going assessment, analysis, planning and implementation of nursing care. Outlined in this chapter is the statement of evaluation, amendment of nursing care plan and termination of care rendered to Mr. Edward Owana.

5.1 STATEMENT OF EVALUATION

Goals outlined in the nursing care plan were met in line with the objective criteria.

The goals fully met included the following:

- Patient was able to breath normally within 2 hours as evidenced by patient's chest was rising and falling at normal rhythm and respiratory rate of 22 breaths per minute.
- Patent's fluid volume returned to normal levels during the period of hospitalization as evidenced by, patient having good skin turgor and blood pressure of 140/90mmHg.
- Patient was relieved of abdominal pain as evidenced by, patient verbalizing absence of pain within 24 hours of nursing intervention
- Patient had patent airway for effective breathing as evidenced by patient verbalising that he had relief from cough on Sunday.

- Patient was able to engage in active exercise during stay on the ward as evidenced by patient being observed to walk to the washroom for elimination and bath without reporting feeling of tiredness
- Patient was able to eat well by the time of being discharged from the ward as evidenced by patient consuming more than 80% of all meals served
- Patient and his wife had adequate knowledge about disease condition by the time of discharge as evidenced by patient and his wife being able to answer questions correctly about the disease process, treatment and prevention of recurrence

5.2 AMENDMENT OF CARE PLAN FOR PARTIALLY MET OR UNMET OUTCOME CRITERIA

During care for Mr, Edward, all objectives set as outcome criteria were fully met. Therefore there was no need to amend the care plan used for his care. Education on the cause of the condition and the need to ensure personal hygiene was however repeated several times until patient was able to give good feedback about the education given to him concerning the cause, management and prevention of gastroenteritis and pneumonia.

5.3 TERMINATION OF CARE

This is the last phase of the nurse -patient relationship. It is a gradual process which started on the day of my first interaction with Mr. Edward and his wife. From the first day, I made them aware that our association would just be temporal. I also made them aware that the relationship will formally end after the last follow up visit. On the last follow-up visit, I reminded Mr. Edward that it was the last day of our association and thanked him for their co-operation. I emphasised the need for personal and environmental hygiene as well as proactive health seeking behaviour as a way of maintaining health and prevent the recurrence of the condition.

SUMMARY AND CONCLUSION

This Patient/Family Care Study has given an account of how the nursing process approach was used in nursing Mr Edward Owana who was admitted to medical ward II of the Korle- Bu Teaching hospital with acute left lobar pneumonia after three days of detention at the Surgical Medical Unit for Gastroenteritis.

He was admitted on the 17th November and discharged on 22nd November. He came with difficulty in breathing, inadequate hydration, weakness, chest pains and intermittent abdominal pains but was cured of all these by the time of discharge. No complications developed. Home visits were done to continue care of patient and his family and emphasise education about condition and importance of hospital review attendance. The care was finally terminated when the client was handed over to the public health nurse

The writing of this care study has been beneficial to me and has allowed me to gain an in depth knowledge about gastroenteritis and pneumonia and their nursing management. Particularly, the writing of this report has improved my report wring skills for several nursing procedures. It has also provided me with adequate knowledge about the nursing process approach and how to use it effectively.

It has enabled me to understand the unique essence of the care study and nursing profession which is “to assist the individual sick or well in the performance of those activities contributing to health or its recovery (or to peaceful death) that he would perform unaided if he had the necessary strength, knowledge or will, and to do so in such a way as to help him regain independence as quickly as possible.— Virginia Henderson, 1966

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SIGNATORIES

NAME OF DOCTOR.....

SIGNATURE.....

DATE

NAME OF CLINICAL SUPERVISOR.....

SIGNATURE.....

DATE.....

NAME OF SUPERVISING TUTOR.....

SIGNATURE.....

DATE.....

NAME OF CANDIDATE.....

SIGNATURE.....

DATE.....

SIGNATURE OF HEAD OF INSTITUTION.....

DATE.....

